Bioline Laboratory
No.38/2, GR Towers,
New Scheme Road,
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SID No : **07033953**
Branch : **POLLACHI**
Mrs. MOHANA PRIYA V
Age / Sex : 26 Y 10 M / Female
Ref. By : **Dr. ARULSELVAN V MD.,DM.,(Neuro).,MRCP., (UK).,FRCP(Lond).,FRCP**

Patient ID : 0700364534
Collected Date : 18/11/2023 / 11:24
Received Date : 18/11/2023 / 14:20
Reported Date : 22/11/2023 / 15:16

Final Test Report

Specimen	Test Name	Result	Units	Reference Range / Method
IMMUNOLOGY				
NMO with MOG Antibody Profile				
Serum/CSF	Myelin Oligodendrocyte Glycoprotein (MOG) Ab	NEGATIVE		NEGATIVE (Immunofluorescence)

Indirect Immunofluorescence with sample screening dilution 1:10. The test detects IgG antibodies against MOG using transfected HEK2 cells as substrate.

- Diagnosis of inflammatory demyelinating diseases (IDD) with similar phenotype to neuromyelitis optica spectrum disorder (NMOSD), including optic neuritis (single or bilateral) and transverse myelitis
- Diagnosis of autoimmune myelin oligodendrocyte glycoprotein (MOG)-opathy
- Diagnosis of neuromyelitis optica (NMO)- Approximately 80% of patients fulfilling 2006 Wingerchuk criteria for neuromyelitis optica are seropositive for aquaporin-4 (AQP4)-IgG. Of the remaining 20%, one-third harbor MOG-IgG. Seropositivity predicts a relapsing phenotype and warrants immunosuppressive therapy. Patients only rarely harbor both antibodies
- Distinguishing NMOSD, acute disseminated encephalomyelitis (ADEM), optic neuritis, and transverse myelitis from multiple sclerosis early in the course of disease
- Diagnosis of ADEM (acute disseminated encephalomyelitis)
- Prediction of a relapsing disease course

Serum/CSF	NMO (Aquaporin 4) Neuromylitis Optica Ab	NEGATIVE		Negative (Immunofluorescence)

Indirect Immunofluorescence with sample screening dilution 1:10. The test detects IgG antibodies against aquaporin-4 using transfected HEK2 cells & primate optic nerve as substrate.
NMO-IgG are disease-specific autoantibody for neuromyelitis optica (NMO) that binds selectively to aquaporin-4 (AQ4), an astrocytic water channel protein. This autoantibody is seen in 73% of Neuromyelitis optica and implicated in its aetiopathogenesis due to its cytopathic effect. It is considered as a specific biomarker to differentiate Neuromyelitis optica (NMO) or Devic`s disease from Multiple sclerosis(MS) and related disorders like relapsing transverse myelitis.

Verified By
Mr.J.Anandkumar

Dr. R.Kavitha Krishna Ph.D. (Biochemistry)
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Final Test Report

Specimen	Test Name	Result	Units	Reference Range / Method
HAEMATOLOGY				
Complete blood count				
EDTA BLOOD	Haemoglobin	12.5	g/dL	12 - 15 (Non-Cyanide Haemoglobin Analysis)
EDTA BLOOD	Total WBC count	**13490** ↑	cells/cumm	4000 - 10000 (Fluorescent Flow Cytometry)
Differential Count				
EDTA BLOOD	NEUTROPHILS	**83.10** ↑	%	40 - 70
EDTA BLOOD	LYMPHOCYTES	**12.20** ↓	%	20 - 45
EDTA BLOOD	EOSINOPHILS	**0.10** ↓	%	1 - 7
EDTA BLOOD	MONOCYTES	4.60	%	2 - 7
EDTA BLOOD	BASOPHILS	0.00	%	less than 1
EDTA BLOOD	PCV	37.0	%	37 - 47 (Electrical Impedance)
EDTA BLOOD	Red Blood Cell (RBC) Count	4.42	million/cumm	3.8 - 4.8 (Electrical Impedance)
EDTA BLOOD	MCV	83.6	fl	83 - 101 (Calculated)
EDTA BLOOD	MCH	28.2	pg	27 - 32 (Calculated)

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Mr.S.Anandan

Dr.R.Karthick Prabhu MD., (Path)
Consultant Pathologist

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Final Test Report

Specimen	Test Name	Result	Units	Reference Range / Method
EDTA BLOOD	MCHC	33.7	g/dL	31.5 - 34.5 (Calculated)
EDTA BLOOD	Platelet count	409000	cells/cumm	150000 - 410000 (Electrical Impedance)
EDTA BLOOD	Absolute Neutrophils	**11210** ↑	cells/cumm	2000 - 7000 (Automated)
EDTA BLOOD	Absolute Lymphocytes	1646	cells/cumm	1000 - 3000 (Automated)
EDTA BLOOD	Absolute Eosinophils	**13** ↓	cells/cumm	20 - 500 (Automated)
EDTA BLOOD	Absolute Monocytes	620	cells/cumm	200 - 1000 (Automated)
EDTA BLOOD	Absolute Basophils	**0** ↓	cells/cumm	20 - 100 (Automated)
EDTA BLOOD	RDW - CV	11.7	%	11 - 16 (Automated)
EDTA BLOOD	RDW - SD	41.1	fl	37 - 54 (Automated)
EDTA BLOOD	PDW	12.2	fl	9.0 - 17.0 (Automated)
EDTA BLOOD	MPV	10.5	fl	9 - 13 (Automated)
EDTA BLOOD	PCT	**0.43** ↑	%	0.17 - 0.38 (Automated)

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Received Date : 18/11/2023 / 09:36
Reported Date : 22/11/2023 / 15:16

Final Test Report

Specimen	Test Name	Result	Units	Reference Range / Method
BIOCHEMISTRY				
Fluoride	Glucose, Fasting	**112.2** ↑	mg/dL	Healthy Adult or children : less than 100 (Colorimetric : GOD - PAP)
Serum	Creatinine.	0.63	mg/dL	0.6 - 1.1 (Colorimetric : Alkaline picrate)
Lipid Profile				
Serum	Cholesterol, Total	**246.1** ↑	mg/dL	Desirable : <200 Borderline high : 200 - 239 High : >239 (Enzymatic : CHOD-PAP)

Note: Above Biological interval is based on 9 to 12 hours fasting.

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Patient ID : 0700364534
Collected Date : 18/11/2023 / 08:10
Received Date : 18/11/2023 / 10:41
Reported Date : 22/11/2023 / 15:16

Final Test Report

Specimen	Test Name	Result	Units	Reference Range / Method
Serum	Triglycerides	80.30	mg/dL	0-9 yr Acceptable: <75 Borderline: 75-99 Abnormal : >=100 10-19 yr Acceptable: <90 Borderline: 90-129 Abnormal : >=130 Adult (NCEP ATP-III) Normal : <150 High : 150 - 199 Hypertriclyceridemic: 200 - 499 Very High : >499 (Enzymatic Colorimetric : GPO-PAP)
Note: Above Biological interval is based on 9 to 12 hours fasting				
Serum	Cholesterol, HDL	53.6	mg/dL	Adult (NCEP ATP-III) Low : < 40 High : >=60 (Direct)

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Final Test Report

Specimen	Test Name	Result	Units	Reference Range / Method
Serum	Cholesterol, LDL	**176.4** ↑	mg/dL	Optimal : <100 Near or above optimal : 100 - 129 Borderline high : 130 - 159 High : 160 - 189 Very high : >190. (Calculation)
Serum	Cholesterol, VLDL	16.1	mg/dL	Less than 30 (NCEP ATP-III) (Calculation)
Serum	Cholesterol/HDL Ratio	4.6		Castelli's Risk Index -I Ideal : <3.5 Good: 3.5-5.0 High: >=5 (Calculation)
Serum	LDL/HDL Ratio	3.3	Ratio	Castelli's Risk Index -II Ideal : <2.0 Good: 2.0-5.0 High: >=5 (Calculation)
Serum	Non - HDL Cholesterol	**192.5** ↑	mg/dL	Adult (NCEP ATP-III) Optimal : <130 Near or above optimal : 130-159 Borderline high : 160-189 High : 190-219 Very high : >220 (Calculation)

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Final Test Report

Specimen	Test Name	Result	Units	Reference Range / Method
Serum	HDL/LDL Ratio	0.3	Ratio	Optimal : >0.4 Moderate: 0.3-0.4 High : <0.3 (Calculation)
Serum	Sodium.	141	mmol/L	136 - 145 (Ion Selective Electrode)
Serum	Potassium.	4.3	mmol/L	3.5 - 5.1 (Ion Selective Electrode)

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Final Test Report

Specimen	Test Name	Result	Units	Reference Range / Method
IMMUNOLOGY				
Serum	25 Hydroxyvitamin D	**15.3** ↓	ng/ml	Deficiency : <= 20 Insufficiency: 21 - 29 Sufficiency : >= 30 (ECLIA)

Comments : Vitamin D is a fat-soluble steroid hormone precursor that is mainly produced in the skin by exposure to sunlight. Vitamin D is biologically inert and must undergo two successive hydroxylations in the liver and kidney to become the biologically active 1,25 - dihydroxyvitamin D. It is commonly agreed that 25-hydroxyvitamin D is the metabolite to determine the overall vitamin D status as it is the major storage form of vitamin D in the human body. This primary circulating form of vitamin D is present human body with levels approximately 1000 fold greater than the circulating 1,25-dihydroxyvitamin D. The half-life of circulating 25-hydroxyvitamin D is 2-3 weeks.
Vitamin D is essential for : Bone health. In children, severe deficiency leads to bone-malformation, known as rickets. Milder degrees of insufficiency are believed to cause reduced efficiency in the utilization of dietary calcium.
Vitamin D deficiency causes : Muscle weakness in elderly, the risk of falling has been attributed to the effect of vitamin D on muscle function. Vitamin D deficiency is a common cause of secondary hyperparathyroidism. Elevations of PTH levels, especially in elderly vitamin D deficient adults can result in osteomalacia, increased bone turnover, reduced bone mass and risk of bone fractures. Low vitamin D (25-OH) concentrations are also associated with lower bone mineral density.
The results should always be assessed in conjunction with the patient's medical history, clinical examination and other findings.

Serum	TSH	1.190	uIU/ml	Adult : 0.270 - 5.350 ATA Guidelines for Healthy Pregnant Women First Trimester : 0.1 - 2.5 Second Trimester: 0.2 - 3.0 Third Trimester : 0.2 - 3.0 (ECLIA)

Note: TSH has a diurnal rhythm, peaks at 2.00-4.00 am and has lowest level at 5.00-6.00 pm with ultradian variation. Hence thyroid test is only a snapshot of what is occurring within a dynamic system and for treatment purpose, the results should be accessed in conjugation with patient medical history, clinical examination & other tests/finding for confirmation. Many multivitamins (such as Vit B7), supplements (especially hair, skin, and nail) and over-the-counter and prescription medications may affect thyroid test results, and their use should be discussed with the healthcare practitioner prior to testing.
When a high serum TSH concentration and normal free T4 is found, repeat measurement 3-6 months later along with thyroid antibodies after excluding nonthyroidal illness and drug interference is recommended.

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Specimen	Test Name	Result	Units	Reference Range / Method
Serum	VITAMIN B 12	250	pg/ml	197 - 771 (ECLIA)

Note: Vitamin B12 or cyanocobalamin, is a complex corrinoid compound found exclusively from animal dietary sources, such as meat, eggs and milk. It is critical in normal DNA synthesis, which in turn affects erythrocyte maturation and in the formation of myelin sheath.
Vitamin-B12 is used to find out neurological abnormalities and impaired DNA synthesis associated with macrocytic anemias. The test results should always be assessed in conjunction with the patients medical history, clinical examination and other findings.

Verified By
Mr.S.Eswaran

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---------- End of the Report ----------

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